



WORKPLACE SAFETY AND INSURANCE APPEALS TRIBUNAL

DECISION NO. 231/12

BEFORE: A.G. Baker: Vice-Chair

HEARING: January 31, 2012, at Hamilton
Oral

DATE OF DECISION: February 7, 2012

NEUTRAL CITATION: 2012 ONWSIAT 266

DECISION(S) UNDER APPEAL: WSIB Appeals Resolution Officer (ARO) June 19, 2009

APPEARANCES:

For the worker: M. Grimaldi, Office of the Worker Adviser

For the employer: Not Participating

Interpreter: n/a

REASONS**(i) Introduction and Issues**

- [1] The worker appeals the decision of Appeals Resolution Officer (ARO) G. Ciccocelli, dated June 19, 2009. From that decision, the worker appeals the denial of ongoing entitlement for the left shoulder, low back, and left hip.

(ii) Issue

- [2] Ongoing entitlement to benefits for the left shoulder, low back and left hip beyond January 2006, claimed to be related to the slip and fall accident of June 24, 2005.

(iii) Background

- [3] As the ARO noted as of December 2008, the worker was 60 years old. She had joined the accident employer in September 2004 as a housekeeper and could be assigned to a number of properties. She was noted to have been assigned largely to a night shift position at one property until June of 2005, when she was successful in obtaining a day shift placement at another location. As the worker was transitioning, she was working at both locations. The worker claimed that she slipped and fell in a Jacuzzi tub on June 24, 2005, while performing her duties at her new placement. The worker attempted to report the injury on the date of the accident, but there was a delay due to the absence of supervisory/management or security staff.
- [4] The worker then missed time from work in her night shift position. The accident employer maintained that the worker was contacted regarding a June 20, 2005 absence, for which there was no explanation. The employer indicated that the absence was reported initially due to a non-work related low back problem. The worker was subsequently terminated for failure to report work absences. The employer also challenged the work relatedness of the claimed injuries.
- [5] After making inquiries, the Board noted that the worker had prior left shoulder, low back, and left hip problems, as well as other difficulties. The Board accepted the June 24, 2005 incident caused an aggravation of the worker's prior left shoulder, low back and left hip problems. Entitlement to benefits was granted on an aggravation basis for the acute phase only. The worker continued with treatment and Loss of Earnings (LOE) benefits were granted as the worker had been terminated from the accident employer.
- [6] The worker was awaiting medical updates to determine the level of the workplace injuries, when she was involved in a January 2006, non-work related motor vehicle accident (MVA). Based on the medical reporting, the Board concluded that the aggravations caused by the workplace accident had ceased, and that ongoing problems were the result of the non-work related MVA. Entitlement to benefits was therefore limited to January of 2006.
- [7] The worker objected to the limitation on the worker's benefits and it was submitted that the evidence supported the aggravation had not resolved. An additional review by the Board resulted in a confirmation of the closure of benefits, and the matter was referred to the Board Appeals Branch. The ARO also denied the worker's appeal and concluded that the evidence did reflect cessation of the June 24, 2005 workplace aggravations. Ongoing difficulties were found

to be more reasonably related to symptomatic pre-June 24, 2005 conditions and/or post-January 14, 2006 events/conditions. The worker has appealed those findings to the Tribunal.

- [8] The following file and medical information was also noted by the ARO. There was no dispute raised by the worker's representative with that information, which stated as follows:

File documentation:

- Worker's report of injury (July 8, 2005) indicated the worker had no prior similar injury or condition
- Field investigation notes (March 2007):
 - o Worker's statement:
 - Claimed ongoing entitlement for June 24, 2005 related left-sided low back and left hip
 - Although initial June 24, 2005 injuries included left shoulder, no ongoing left shoulder problems
 - Denied pre-June 24, 2005 workplace injury
 - Denied pre-June 24, 2005 left hip problems
 - Confirmed a non-work MVA in 1981 or 1983, which involved low back and cervical injuries
 - MVA related settlement of approximately \$110,000.00 in 1996
 - Was employed at the time of the 1981/83 MVA
 - Unable to work following the 1981/S3 MVA, due to the MVA related injuries
 - Post-MVA attempts to return to work were discontinued due to MVA related injury discomfort
 - MVA related medical/legal documentation was unavailable
 - Sustained complete recovery of MVA related injuries shortly after the MVA claim settlement
 - Unable to recall employment history between MVA and employment with the AE
 - Did not pursue employment between MVA and AE, due to elder and grandchild care responsibilities
 - Denied a June 20, 2005 low-back home injury or work absence from either position
 - Confirmed a non-work MVA January 14, 2006 (rear-ended)
 - Hurt shoulder in MVA
 - Low-back and hip problems pre-dated the January 14, 2006 MVA
 - Uncertain if January 14, 2006 MVA worsened low-back and hip condition
 - No post-AE employment
 - Did not qualify for MVA related insurance benefits due to workplace injury/benefits
 - Worker left a note for AE June 24, 2005, advising of the anticipated June 25, 2005 absence
 - Co-workers aware of workplace injury and worker's reporting attempts
 - Supervisor called June 25, 2005 regarding worker's absence

- Human Resource representative June 27, 2005 contact regarding worker's absence and request for supporting medical
- Medical note provided to the AE June 28, 2005, when AL terminated the worker
- o A/E:
 - Worker commenced training in the new position on June 15, 2005, and continued working the old position
 - Worker attended the new job twining June 19 & 20, 2005, in respect to the old job the worker was late June 19, 2005 and absent June 20, 2005
 - Worker arrived late for June 23 & 24, 2005 shifts
 - Contacted the worker June 20, 2005 regarding the work absence, and worker advised of low back pain due to a home incident
 - Decision to terminate worker was due to worker's failure to comply with employment requirements regarding absence, tardiness, and appropriate job change notice
 - Denied worker's reporting of a workplace injury until June 28, 2005
 - No record of worker having prior similar problems (left shoulder, low back, left hip)
 - Worker had an alternate/pre-June 2005 workplace injury/claim; therefore worker was aware of reporting protocol
- o Co-worker:
 - [Co-worker]:
 - Aware of worker making complaints of general aches and pain, but non-specific
 - Unaware of worker having had a June 2005 work-related injury
 - [Co-worker]:
 - Aware worker missed a shift due back pain and medication impacts
 - Aware of worker's reported tub slip/fall incident
- o 2006 MVA related insurance:
 - Worker did not meet threshold for income replacement benefits since worker was not working prior to the MVA
 - Worker was granted entitlement for related health care expenses

Medical:

- Family doctor chart notes were received March 6, 2007
- Family doctor chart notes showed numerous non-work ailments (November 2002 to February 2007) as well as:
 - o November 13, 2002
 - Left trochanteric bursitis - no onset history
 - o November 27, 2002
 - Left hip pain recurring
 - Injection helped 80%

- o March 12, 2003
 - Follow-up left hip, referred for orthopaedic assessment
 - Appointment with Dr. Offerski - May 1, 2003
- o January 22, 2004
 - Chest pain radiating into left upper limb with numbness in left hand.
 - Mild shortness of breath
 - History of H.H., cervical spine problems, family history of I.H.D.
 - Mild tenderness cervical spine — range of motion normal
 - Left shoulder/neck - negative
- o January 29, 2004
 - Cervical X-ray — moderate degenerative disc disease
- o April 3, 2004
 - Steroid injection for left hip — without flare-up history
 - Medication
- o June 11, 2004
 - Left hip pain
 - Vioxx not helping
 - Worse when stands or walks
 - Refer for X-ray
- o August 13, 2004
 - Left hip pain getting worse despite medication and steroid injection
 - X-ray left hip and lumbosacral spine
 - Needs orthopaedic consultation
- o September 1, 2004
 - X-ray of lumbosacral spine showed mild degenerative disc disease
 - Conservative treatment
 - Already on Vioxx
 - Left hip X-ray — most likely bursitis
 - When flares up - limited standing, walking
 - Hot baths
- o September 29, 2004
 - Fell at work yesterday, hit face
 - Pain and headaches
 - Abrasion on nasal bridge
 - Tenderness over left shoulder, restricted abduction
 - X-rays, ice, medication, and rest

- o October 4, 2004
 - Back to work tomorrow, with restrictions
- o October 15 2004
 - Marked pain left shoulder/arm, right side of neck
 - Pain on movement
 - Diagnosis — tendonitis
 - Medication and physiotherapy
- o December 1, 2004
 - Still marked pain left hip
 - Can't take NSAID's or Tylenol #3, try Percocet
- o January 18, 2005
 - Left hip pain
 - Percocet helps but makes her [worker] doppy
 - Has appointment with orthopaedic specialist in June
- o April 25, 2005
 - Complaints of increasing back pain
 - On examination — good condition
 - No related clinical findings
 - Refill Percocet
- o June 21, 2005
 - Check-up - Doing very well
 - On examination — good condition
 - No related clinical findings
 - Refill Percocet
- o June 27, 2005
 - Slipped/fell at work June 27, 2005
 - Pain low back (left side), left shoulder
 - On examination
 - o Limping
 - o Tenderness over left hip (posteriorly)
 - o Tenderness top of left shoulder, pain abduction & external rotation
 - X-rays
 - Ice, rest (3 days), no lifting, Percocet for pain
- o July 15, 2005
 - Still marked pain
 - Can't stand easily when sits
 - Severe tenderness lumbosacral spine

- Painful flexion/extension of back
- Physiotherapy and refill Percocet
- o August 22, 2005
 - Still going to physiotherapy
 - Ice, heat
 - Refill Percocet
- o September 23, 2005
 - Complaints of pain/stiffness left side of neck, parasthesia left arm
 - Neck - tenderness left trapezius
 - Pain on passive range of motion — flexion & left cuff rotation
 - Diagnosis — musculoskeletal pain
 - Medication
- o October 25, 2005
 - Still complaining of back pain
 - Going to physiotherapy
 - On examination
 - o Not in pain
 - o Not limping
 - o No related clinical findings
 - Refill Percocet
- o January 3, 2006
 - Still complaining of marked back and hip pain
 - On examination
 - o No pain
 - o No limping
 - Refill Percocet
- o January 17, 2006
 - MVA January 14, 2006
 - Didn't go to emergency
 - Reporting headaches, marked pain and stiffness neck and shoulders, left lower ribs
 - On examination
 - o No pain
 - o No limping
 - o Cervical spine — stiff, tender muscles, painful range of motion
 - o Shoulders — very tender, limited abduction, external rotation
 - o Tender left lower ribs (anteriorly) no bruises
 - o No neurological deficit

- Diagnoses
 - o WAD II - Neck strain
 - o Bilateral shoulder strain
 - o Left rib cage strain
- Percocet, physiotherapy
- o February 21, 2006
 - Still complaining of marked pain
 - Wants paper signed for physiotherapy and medication refill — done
 - Needs more sleeping pills — can't sleep because of pain
 - Cramps in right leg when walking, changing in ? toes
 - On examination
 - o Right foot — red toes
 - o Couldn't feel dorsalis pedis pulse
 - Referred for Doppler ultrasound of leg?
- o March 16, 2006
 - Follow-up ultrasound
 - Decreased ABI left side to?
 - Refer to vascular surgeon
 - On examination — no related clinical findings
 - Refill Percocet
- o March 29, 2006
 - Still reporting marked numbness lateral side of left-neck, left scapula/shoulder
 - Going to physiotherapy
 - Need more pain medication and something to help sleep
 - Refill Percocet and by Restoril
- o April 26, 2006
 - Still pain back of neck and between shoulder blades — stiffness
 - Going to physiotherapy
 - Tenderness over back of neck and right trapezius
 - Pain on arm abduction
 - Diagnosis — post-traumatic myofascial pain
 - Continue physiotherapy and Percocet
- o May 10, 2006
 - Check-up, again having bouts of chest tightness
 - Has significant P.A.D.
 - Refer for blood work, U/S, EKG, and Dr. Zimakas

- o June 6, 2006
 - CT scan showed severe stenosis of left common iliac artery
 - Need surgical consult
- o June 19, 2006
 - Reporting marked neck pain causing migraine headaches
 - Musculoskeletal low back pain
- o July 19, 2006 & August 14, 2006
 - Similar complaints and findings
- o October 2, 2006
 - Check-up and refill
 - Did a lot better
 - No related clinical findings
- o November 10, 2006
 - Still reporting marked pain back and left hip
 - Physiotherapy not helping
 - Had by-pass surgery
 - No related clinical findings
- o December 13, 2006
 - Reporting increased neck and left shoulder pain
 - No numbness
 - Percocet causes forgetfulness and fatigue
 - On examination
 - o No pain
 - o Tenderness over left trapezius and left shoulder
 - o Pain on forward flexion of neck and left arm abduction
- o February 26, 2007
 - Check-up
 - Reporting marked pain left hip
 - Saw Dr. LeRoux last week — cortisone injection
 - Gained a lot of weight due to inactivity
 - On examination
 - o No pain
 - o No limping
 - o No related clinical findings

- o Left hip/pelvis and Lumbosacral spine X-rays (August 13, 2004)
 - Mild degenerative changes in the lumbosacral spine
 - Arthrosclerotic calcifications in the abdominal aorta
 - No abnormality in the left hip joint
- o ACT Health (March 8, 2005) discharge report:
 - Worker could independently continue with a home exercise program
- o Dr. Zimakas (June 21, 2005)
 - Follow-up of worker for various non-work ailments
- o Left hip/pelvis and Lumbosacral spine X-rays (June 27, 2005)
 - Findings similar to August 13, 2004
- o Dr. Martin — Orthopaedic specialist (August 16, 2005)
 - Worker complaining of lateral left hip pain
 - Related initial onset to a fall in 2004 — with pain beginning months later
 - Exacerbated by another recent fall
 - Anti-inflammatories, Tylenol #3, and physiotherapy provided
 - On examination
 - Both hips - full, equal range of motion
 - Left - marked discomfort in lateral aspect at extremes of internal/external rotation
 - Tender trochanteric area
 - Ilio-tibial back seemed tight
 - X-rays showed hip joint-normal
 - Diagnosis — left trochanteric bursitis
 - Treatment — injection to affected bursae and continued physiotherapy
 - No mention of left shoulder or low back
 - No details of prior similar non-work hip problems or 1981/83 MVA
- o ACT Health (December 20, 2005) discharge
 - Worker did not attend beyond December 13, 2005
 - WSIB limited entitlement
 - Worker to continue with independent home exercise
- o Police report (January 14, 2006) — MVA
 - Minor vehicle damage indicated (\$500.00 each)
 - No indication of personal injuries

- o Application for Accident Benefits (March 8, 2006) — MVA
 - Reported areas of injury
 - Bilateral shoulders
 - Neck
 - Upper back
 - Right foot
- o July 2006 statement to MVA insurer, worker maintained the left hip problem was due to the workplace injury of June 24, 2005 rather than the January 14, 2006 MVA
- o McLarens Canada letter (November 20, 2006) indicated the worker felt the January 2006 MVA aggravated the prior left hip problems
- o MVA insurer provided the worker several assistive devices for home use
- o MVA insurer In-home assessment of worker (January 10, 2007)
 - Worker's reported past medical history included
 - Two prior MVA (1983 and 2000)
 - 1983
 - o Back injury (herniated disc)
 - o Cracked left patella
 - o Required 10 years of therapy.
 - 2000
 - o Struck back of head on head-rest
 - o Chest pain
 - o Pain in back of head
 - o Vehicle damage - \$2,000.00
 - o No lingering injuries
 - Two workplace accidents/injuries (2004 and 2005)
 - o Herniated disc
 - Did not reflect October 2006 left hip surgery (stunt)
 - Did not reflect prior similar non-work ailments (eg. Cervical, Lumbar, shoulder, hip, headaches, and sleep disturbance)
 - Current complaints included:
 - Cervical, thoracic, lumbar spine
 - Bilateral shoulders
 - Left hip
 - Headaches
 - Sleep disturbance
 - Attended physiotherapy March — December 2006
 - Demonstrated inconsistent responses between formal examination and causal worker observation
 - Scheduled to see specialist (February 15, 2006) regarding herniated disc
 - Worker was considered capable of all pre-January 14, 2006 housekeeping activities and did not qualify for non-earner benefits
- o Dr. Bartel le Roux — Orthopaedic specialist (February 15, 2007)
 - Worker complaining of low back pain since July 2005
 - Also pain in lateral left hip
 - Based on X-rays and examination — conclusion

- Trochanteric bursitis
- Signs of lumbar spondylosis with mechanical back pain
- Recommended treatment
 - Physiotherapy
 - Referral to Dr. Magee for spinal cortisone injection
 - Cortisone injection for left hip/trochanteris bursitis
 - MRI of the low back
- No mention of left shoulder
- No mention of the past MVA (1981/83 or 2006)
- o Lumbar MPJ (February 7, 2007)
 - Mild degenerative changes
 - Probable parapelvic cysts in both kidneys — should be confirmed with ultrasound
- Memo #1 & #4 (August 11, 2005 & January 5, 2006, respectively) were concluded without the benefit of the family doctor's chart notes which demonstrated the worker's pre-June 24, 2005 medical status
- Memo #14A & 18A (May 9, 2007 & June 22, 2007, respectively) WSIB medical consultant reviews and comments regarding workplace injury status

(iv) Worker's Testimony

[9] The worker also testified that she had been performing her regular duties on the day of her 2005 accident. She had been cleaning a tub and walls. She slipped and hurt her back, left hip and left shoulder. She stated it was her last room and tried to report the accident. That failed, and she left for the day, subsequently seeking medical attention. She stated she had been with the employer for about a year and had no difficulty with her duties despite pre-existing health problems. Whereas after her accident she had several limitations and ongoing problems. That included no longer being capable of heavier tasks that included lifting and pulling duties.

[10] The worker also admitted she had prior back and shoulder problems. She had fallen in 2004 and had a few days on modified duties. She hit her shoulder and face. She stated that the shoulder also had a diagnosis of bursitis, but remained more problematic after her 2005 work accident. She also stated that it is a lot weaker now. She also noted involvement with both shoulders from her MVA in 2006.

[11] The worker also stated that her low back had been a problem since the early 1980's from an MVA. However, the worker stated that she had learned to live and work with it, and that it was not a problem before her June 2005 work accident. She stated the fall in 2005 aggravated the back again and it was a constant issue since the fall. She stated it was weaker after the fall. The worker also stated that her left hip was similarly impacted by the fall and continues to be a problem.

[12] The worker also addressed her January 2006 MVA. She stated she was rear ended. She stated her main difficulties after the accident were whiplash type injuries to the neck and shoulders, as well as a severe headache from hitting her head. She also stated she injured her ribs. She stated however that her seatbelt caught her and she banged her head, which caused her main areas of injury.

[13] The worker stated that she did not go to emergency after the MVA, as it was a minor accident. She went to her family doctor a couple of days later. She stated she did not think the accident impacted her prior injuries, but she was having cervical difficulties and her shoulders were a problem below her neck bilaterally. The worker was also assessed by an occupational therapist for her private insurer in 2007.

[14] The worker stated she would have returned to her prior job after her June 24, 2005, accident, but she was terminated. She also stated she likely could not have returned to full duties given her aggravations, ongoing treatment, and limitations. In particular, she stated she could not have performed her heavier duties. The worker also noted a substantial increase in medication use after the 2005 work accident, which included increasing pain and anti-inflammatory use from occasional use a couple of times a month, to regular daily use. She was also prescribed sleep medication for regular use.

(v) Law and Policy

[15] The *Workplace Safety and Insurance Act, 1997* (WSIA) is applicable to this appeal. Under Workplace Safety and Insurance Board (the Board) *Operational Policy Manual* (OPM) Document No. 11-01-15, a worker who has a pre-accident impairment and suffers a minor work-related injury or illness to the same body part or system is entitled to benefits on an aggravation basis. Such benefits are generally considered for an acute episode and benefits continue until the worker returns to the pre-accident state. The policy also provides that a worker may be entitled to a Non-Economic Loss (NEL) benefit if he or she suffered a permanent impairment as a result of an aggravation.

[16] Section 46 of the *Workplace Safety and Insurance Act, 1997* (WSIA) provides that if a worker's injury results in permanent impairment, the worker is entitled to compensation for non-economic loss. According to section 2(1) of the WSIA, "Impairment" means a physical or functional abnormality or loss (including disfigurement) which results from an injury and any psychological damage arising from the abnormality or loss. Also according to section 2(1) of the WSIA, "Permanent impairment" means impairment that continues to exist after a worker reaches maximum medical recovery (MMR) (formerly referred to as maximum medical rehabilitation under the pre-1997 Act). Under Board OPM Document No. 11-01-05, MMR is reached by a worker when it is not likely that there will be any further significant improvement in their medical condition.

[17] I also note Board OPM Document No. 11-02-02, entitled "Lost Time Claims", as well as Board OPM Document No. 18-03-02, entitled "Payment of LOE Benefits", which provides that a worker who has a loss of earnings as a result of a work-related injury is entitled to payment of LOE benefits.

(vi) Decision

[18] In this case, I noted the nature of the worker's pre-existing conditions. In particular, that the worker's left shoulder, low back, and left hip had been symptomatic prior to her June 24, 2005 accident. She would have flare-ups of her conditions. I also noted the Board Medical Consultation from Dr. Connors in May and June of 2007. The doctor noted that the

insurance records from the January 2006 MVA indicated ongoing neck and bilateral shoulder strain. There was no indication of ongoing symptomatic low back pain.

[19] Dr. Connors also noted the prior family doctor history on file. That included a bursitis diagnosis and left hip difficulties in 2002 to 2004. It was evident the worker was having some left hip pain and had a steroid injection in April of 2004. Given the worker's age, late 50's at the time of the accident, it was also not surprising to see degenerative problems indicated for the lumbar spine from x-rays prior to the work accident.

[20] Dr. Connors also noted the worker's condition after her aggravation and that she was continuing to have back and hip problems in late 2005 and early 2006. The doctor also noted the worker's MVA. While the left lower back was cited, the diagnoses focused on the worker's whiplash injury to the neck and bilateral shoulders. Further, it was evident from a July 2006 report that the worker did have further neck, low back, and sleep problems, as well as headaches. Further testing into 2007 also again noted the worker's bursitis and degenerative difficulties to the low back.

[21] Dr. Connors concluded, and I also recognize, that the worker was suffering bursitis and degenerative difficulties, as well as osteoarthritis that pre-existed the work accident. The doctor also found that the work accident history was consistent with aggravation of the hip and low back conditions. The doctor however found that the worker had achieved a medical plateau before her MVA in 2006, and that there was no evidence of residual impairment from her 2005 work accident.

[22] However, I also noted the doctor's comments that "this is a very difficult file to review and come to any clear conclusion. There has been a number of difficult issues around the work event history ..." I took that comment as indicating that it would be difficult to clearly conclude that the work accident was not related to any of the worker's ongoing difficulties.

[23] That said, I turn first to the worker's shoulder claim. It was evident that there was indication it may not have fully recovered before the MVA. However, I was persuaded that the combination of pre-existing difficulties, and the shoulder injuries from the MVA, substantially impacted the worker's left shoulder. Again, I note the worker had substantial involvement of the bilateral shoulders from her MVA. It was also evident for example from clinical notes that the worker had prior "marked pain" in the left/shoulder and arm in October of 2004. In my view, the combination of pre-existing difficulties and the impact of the 2006 MVA were very significant. Therefore, I find that the 2005 work accident was not a significant contributing factor to the worker's ongoing left shoulder condition, and further benefits beyond January 2006 are denied in that regard.

[24] It was also evident that the worker had pre-existing left hip and low back difficulties. They had also been cited in a number of prior clinical records noted above. However, it was evident that neither the left hip nor the low back problems were significantly impacted by the 2006 MVA, which had been described as a minor accident. Rather, the worker's neck, shoulders, upper back and right foot were the reported areas of injury in her application for accident benefits. As such, I was not persuaded noting the above information that the MVA was of significance in regard to the worker's low back or left hip.

- [25] Further, despite the noted pre-existing left hip and low back difficulties, the worker had been with the employer for almost a year and performed her regular duties for the bulk of the time leading up to her 2005 accident. She had also been seen by her family doctor three days before her June 2005 work injury, and had been found to be doing very well with no related clinical findings. The worker then has her work accident and the Board accepts that she suffered aggravations of those pre-existing conditions.
- [26] The aggravations continued with the worker taking markedly increased medication and attending physiotherapy. She did not return to work and testified that even if she had, she would not have been capable of performing her duties as before her work accident. As noted above, the worker had ongoing complaints of back pain and hip pain during the intervening months between the work injury and the 2006 MVA. It was also evident that those conditions, while potentially having reached a plateau, did not in my view return to their pre-accident state.
- [27] I noted in particular that the worker was seen by her family doctor on January 3, 2006, shortly before her MVA. She was still complaining of marked back and hip pain. It was also notable that the worker continued with physiotherapy and a discharge report was noted on December 20, 2005. It was noted that there was no return to work recommended on either full or modified duties, and that the worker was to continue independently with exercises. That in my view does not support a worker reaching maximum medical rehabilitation that is consistent with her pre-accident state. Rather, it was evident the worker remained unable to return to work, and it was evident that there was residual impairment.
- [28] It was also evident from the medical information, such as the family doctor clinical notes, that the worker's back and hip continued to be problematic into late 2006 and into 2007. I also noted reporting from an occupational therapist in 2007 that the worker's aggravated conditions had continued. As noted from the findings of the therapist in February 2007, almost two years post-accident, that the worker remained in need of modifications in order to safely perform daily living activities, along with assistive devices being recommended. In that regard, complaints were focused not only on the injuries from the MVA, but also on her pre-existing low back and left hip problems. In my view, the report indicated the worker continued to have ongoing difficulties in those areas at an evidently reduced level from her pre-work accident state.
- [29] As such, I find that the worker is entitled to further benefits for her work injuries to the left hip and low back beyond January 2006. That includes NEL assessments for permanent aggravations of those conditions. The Board will adjudicate the nature and duration of further benefits once those assessments have been completed.

DISPOSITION

[30] The worker's appeal is allowed in part.

[31] The worker is not entitled to further benefits for her left shoulder aggravation.

[32] The worker is entitled to further benefits beyond January 2006 for permanent low back and left hip impairments, including NEL assessments for those injuries. The nature and duration of benefits beyond January 2006 is remitted to the Board for further adjudication, subject to the usual right of appeal.

DATED: February 7, 2012

SIGNED: A.G. Baker